



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

Patient leave and escort – Mental Health

Reference #: FSFH-MENH-POL-0022

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital Cockburn Health	Mental Health Service	Medical, Nursing, Allied Health

1.Introduction

Leave for admitted mental health patients is encouraged to support person-centred recovery, sustain community-based living and to ensure treatment is provided in the least restrictive manner possible. There may be times when mental health inpatients are required to attend external appointments with an escort e.g. court appearance. This document outlines requirements to be met when either granting leave or facilitating an escort to mental health inpatients to ensure the safety of both the patient and others.

2.Terminology

Voluntary patient	A person who is proposed to be, or is being treated, by a mental health service voluntarily.
Involuntary patient	A person who is under an involuntary inpatient treatment order or an involuntary community treatment order Mental Health Act (MHA) 2014 s.21
Personal Support Person (PSP)	Can be any of the following people who are supporting someone who is experiencing mental illness - close family member, carer, nominated person, the parent or guardian of a child, any guardian or enduring guardian of an adult (MHA 2014 s.7).
Escort	A patient's carer or PSP, other identified person, or clinician of any profession or mental health worker who accompanies a patient to an appointment, destination or outing.
Mental State Examination (MSE)	A structured psychiatric assessment divided into categories of appearance, behaviour, motor activity, speech, mood, affect, thought process, thought content, perceptual disturbances, cognition, insight, and judgment. This information is used as part of the diagnosis and monitoring of mental illness and risk.

Absent person	Any voluntary mental health patient whose whereabouts should be known but cannot be ascertained after diligent efforts to locate and/or contact them have been made by staff. However, the patient's life or health is not believed to be in danger, and they are not believed to be at risk of harming others.	
Missing person	The Chief Psychiatrist defines a 'missing person' as 'Any voluntary psychiatric patient at high risk of harm who is missing from a mental health service, general hospital or emergency department, without the agreement of or authorisation by staff.' 'Harm' includes harm to self, harm to others and harm to property, such as arson.	
Absent without leave	When an involuntary person is absent without approved leave (AWOL) from a hospital or other place (MHA 2014 s.97).	
Emergency plus™ application		Community based emergency system endorsed by the FSFHG Mental Health Service. It can be accessed by downloading the application onto a work issued Smart phone. Enables staff working with patients outside the hospital or clinic to contact emergency services and accurately communicate their location.

3. Policy

- On inpatient admission:
 - the consultant psychiatrist, in consultation with the treating team will decide and document the leave status of involuntary patients.
 - voluntary inpatients may leave the hospital premises, however, are encouraged to stay on the ward until their first Consultant review.
- Potential risks of leave will be weighed against the positive benefits of empowering the patient to take more responsibility and control of their life recognising the cultural, social, and individual needs of the patient and family/significant other. Cultural leave will be considered where appropriate.
- Treating team decisions to support or decline leave requests will be based on assessment of patient's clinical presentation, consideration of the existing care plan, recent risk assessment and management plan and capacity for informed consent and decision-making.
- Decisions by the treating team regarding leave will be made in partnership with the patient and their PSP having regard for the patients and carer's/family's and PSPs needs, wishes and preferences, and actively considering immediate and residual stressors.
- The persons age and developmental status must be taken into account when granting leave with consideration of sexual safety.

- For patients aged 16-17 years the Treating Team or On Call Consultant will gain consent from the legal guardian for leave unless the patient is deemed a mature minor.
- The risk formulation should include early assessment and regular reassessment of the risk of the patient going missing or becoming AWOL.
- A MSE and risk assessment, rationale for leave and any identified safety planning strategies must be documented in the patient's medical record at the earliest opportunity prior to leave commencing.
- AWOL alerts must be handed over each shift and documented:
 - in PSOLIS
 - Risk Assessment Management Plan (RAMP)
 - in handover iSOFT
- Where leave is restricted, the patient will be advised when the leave restrictions will be reviewed. The type and timeframe of staff responses required should the patient abscond will be clearly documented.
- If the patient has impaired decision-making capacity a substitute decision maker is to be identified for the purpose of leave management. Refer to Advance Health Directives, Advance Care Plans and Enduring Power of Guardianship policy. Document details of decision making in the patient's medical record.
- Safety planning should, wherever possible, be:
 - implemented in partnership with the patient and family / significant other.
 - documented in the patients' medical record.
 - easily accessible by staff.
- Regardless of the legal status of the patient under the Mental Health Act (MHA) 2014, the allocated nurse will review and document the patient's current risk assessment and mental state before each episode of leave and request the treating Medical Officer review when indicated and document in the patient's medical record.

4.Types of leave

- Leave on the grounds of the hospital may be:
 - unescorted grounds access (UGA)
 - escorted grounds access (EGA).
- Leave off hospital grounds may be granted as unescorted or escorted leave.

- The treating team will determine when an escort is required for either ground access or off hospital ground's locations based on clinical need of the patient and their risks.

5. Granting of leave

5.1. Voluntary patients

- On admission staff will inform the patient of local leave processes including signing the leave log sheet and the general responsibilities held by both voluntary patients and staff for supporting treatment adherence and safety during leave.
- Patients and family/significant others are encouraged to inform staff when the patient is taking leave.
- Staff will ensure patients and family/significant others are provided an opportunity to discuss leave arrangements and safety planning prior to leave.
- All leave from the ward must be documented in the patient's medical record.

5.2. Involuntary patients

- Involuntary patients may only be granted leave by a Consultant Psychiatrist:
- The decision to grant leave will consider risks and include the views of the patient, family/significant others and the multidisciplinary team (MDT) and whether voluntary status or a Community Treatment Order (CTO) would be more appropriate.
- All episodes of leave (day leave and overnight) must be documented on a 'Form 7A Grant of Leave to an Involuntary Patient' in PSOLIS and in the medical record.
- Leave length may range from minutes to several days. When leave is longer than three days a MDT review will determine if discharge is more appropriate.
- The patient's PSP must be notified of any grant, variation or cancellation of leave and this must be documented in the patient's medical record.

5.3. Declining granted leave

- Mental health clinicians may exercise their discretion to decline leave that has been granted, pending review by the Treating Team/ DMO if a clinical assessment identifies the presence of any or all of the following factors:
 - there has been an increase in risk and/or stressors.
 - there has been an adverse change in the patient's mental state or physical condition.

- it is greater than 24 hours since the patient's last psychiatric medical review.
- Voluntary patients will be encouraged to remain on the ward pending medical review.
- All details regarding the situation, the factors identified and the rationale for declining approved leave are to be documented in the patient's medical record by nursing and medical, discussed with the patient and family/significant other and handed over to the Multidisciplinary team.

5.4. Revoking leave

- Leave may be revoked by the Consultant Psychiatrist prior to or during leave. The rationale for revoking approved leave must be documented in the patient's medical record.
- Inform the patient's PSP when leave has been revoked.
- Patients who have had leave revoked and remain on the ward should be considered for increased observations.

6. Absent, Missing, AWOL patients.

6.1. Voluntary patients

- If a voluntary patient does not return from leave, or is absent from the ward, staff will:
 - attempt to contact the patient to establish their safety.
 - immediately report the patient's absence to the Ward Coordinator/ Clinical Nurse Specialist or Nurse Unit Manager and the responsible Treating Team.
 - treating team to review current assessed risk and determine whether the patient is considered 'absent' or a 'missing' person (as per the Terminology) and documenting actions to be taken in the patient's medical record.
 - communicate, collaborate and share information with carers/family and PSP.
 - inform the patient's Care Coordinator if they are currently active with a Community Treatment Team. Refer to SMHS Discharge Against Medical Advice or FSFHG Patient initiated Early Discharge.
 - the treating team may decide to discharge the patient who is not considered to be a risk to themselves or others.

- report missing person incidents (or near miss) in accordance with the [MP 0122/19 Clinical Incident Management Policy](#)

6.2. Involuntary patients

- If an involuntary mental health inpatient on leave does not return at the agreed time, does not comply with any conditions attached to the granting of leave or is absent from the ward the patient will be considered AWOL.
- In the event of AWOL:
 - immediately report the patient's absence to the Ward Coordinator/ Clinical Nurse Specialist or Nurse Unit Manager and the responsible Treating Team.
 - undertake appropriate diligent efforts by health professionals and/ or other staff to locate the patient prior to further action being taken.
 - the treating team will assess risk and determine whether a patient is at clear and imminent risk and document the steps to be taken.
 - apply powers under the MHA 2014 in response to consumers who are AWOL and considered at elevated risk, including the role of Transport Officers and/ or WA Police Force, where an Apprehension and Return Order (Form 7D) is made.
 - communicate, collaborate and share information with family/ significant other, peer worker and other agencies involved in the patient's care (where appropriate).
 - Report the AWOL person incident (or near miss) in accordance with the [MP 0122/19 Clinical Incident Management Policy](#)
 - documenting the incidence of AWOL in the patient's medical record.
 - provide care in line with the SMHS [Missing or suspected missing mental health patients](#) policy.

7. Escorted leave

7.1. General considerations regarding all escorts

- At times an escort may be needed to accompany an inpatient to an external appointment or destination off hospital grounds.
- When determining escort suitability, the MDT will consider the designation of the proposed escort and/or the nature of the escort's relationship with the patient and the type of escort itself. The need for the escort to carry medication and if they are authorised for this role will also be considered.

- Information will be gathered from key stakeholders involved with the patient, including family/significant other.
- The patient's next of kin, family/significant other or another identified responsible person may escort the patient at the treating Psychiatrist's discretion subject to them agreeing to the conditions of the leave.
- The patient's preference for an escort (gender, existing therapeutic relationship) will be considered whenever possible unless:
 - the nominated escort does not consent to escorting the patient; or
 - the Psychiatrist is concerned that, based on current risk assessment the nominated escort is likely to be placed in an unmanageable situation during the escort.
- If the Psychiatrist authorises escorted leave, the person(s) considered suitable to provide the escort must be specified by name, role and phone number in the patient's medical record.
- Staff will inform the escort of the expectations of their role including:
 - escort duration and leave conditions.
 - the need to stay with the patient in accordance with their clinical condition.
 - safety planning for period of leave / escort
 - be able to contact the hospital and be contactable via mobile phone.
 - discuss how the escort went with Nursing staff on the patient's return to the ward.
- If the proposed escort does not agree to the above conditions, leave may be withdrawn until alternative arrangements can be made.

7.2. Unpredictable behaviour during staff EGA

- If a serious imminent risk of harm to/ from the patient, a staff member or the public/ bystander, the escort will:
 - ensure their own safety first before taking reasonable steps to ensure the supervision and safety of the patient.
 - activate personal duress alarm to notify Security and contact the SJA, Police for assistance where required.
 - inform the ward via ward mobile phone.
- If a patient attempts to abscond, staff will verbally encourage the patient to return. Refer to the Anticipating and Reducing the Risk of Unpredictable Behaviours Policy.

If a patient attempts to abscond staff will remain with the patient wherever possible however must not follow the patient off hospital grounds.

- Clinician to complete a clinical handover regarding the incident to the treating team.
- Report in Datix Clinical Incident Management System as relevant and document in the patient's medical record.

8. Escorted leave off hospital grounds.

8.1. General considerations

- All proposed escorts will be discussed with and coordinated by the respective Nurse Unit Manager (NUM), Hospital Out of Hours Manager (HOOT) or Shift Coordinator and travel arrangements confirmed in advance.
- When determining the type of patient transport, consideration will be given to:
 - trauma history
 - known risk factors.
 - behavioural concerns
 - timing of appointment and staff availability
 - nominated person/ carer involvement.

Assistants in Nursing and/ or Nursing/ Allied Health Students will not escort a patient unless they are accompanied by another staff member.

- Allied Health including Welfare Officers or Aboriginal Mental Health Liaison may escort a patient if agreed by the Consultant Psychiatrist.
- Nominated PSP/ family/ significant other/ community managed organisations may escort patients off campus if agreed by the treating team.
- Prior to escort confirm travel arrangements including the following details:
 - flights
 - bus pass or
 - taxi voucher
- Prior to leaving the ward the treating team will ensure the following details are documented in the medical record:
 - risk assessment and management plan (RAMP)
 - MSE including physical appearance.

- prescribed medication for the escorted leave period.
- The NUM/ HOOT/ Shift Coordinator will ensure the ward mobile phone is taken on all staff escorts:
 - The ward mobile phone must have the Emergency Plus application installed.
 - The Emergency Plus app will be used as per the FH Duress Alarms and Response and FSH Cockburn Health Code Black and FSH Code Black in Mental Health Unit policies.

8.2. Escort duties

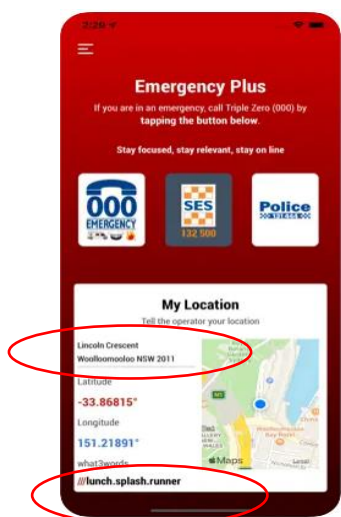
- The Shift Coordinator will discuss any risk factors and appropriate management strategies with the escort prior to leaving the ward and request they inform staff when the patient has returned to the ward and escort outcome. They will be advised to remain with the patient until:
 - they return with them, or
 - hands them over to another person who takes over responsibility for the patients care.
- Maintain visual contact and remain in close physical proximity with the patient where possible.

8.3. Staff escort off hospital grounds

- Staff escorting a patient will ensure:
 - adequate food and fluids are available for the patient for escort duration.
 - prescribed medications are available for the escort.
 - clinical handover is provided as per the FSFHG Clinical Handover policy.
- Any relevant costs incurred whilst a staff member is performing the role of an escort will be reimbursed.
 - To obtain a reimbursement the staff member must complete a Health Support Services Reimbursement of expenses (AP2) or Petty Cash (PC2) form and submit the form to the Line Manager for approval and signature.
- The escorting staff member will contact the Nurse Unit Manager/ HOOT to confirm when they have handed the patient over to the receiving party and are returning to the transferring service.
- The escorting staff member will document details of the escort in the patient's medical record and iSoft handover.

8.4. Unpredictable behaviour off hospital grounds

- Where a patient has been unable to be safely managed whilst away from the ward and absconds the escort will note a description of the patient's clothing and direction of travel.
- Notify the NUM/ HOOT/ Shift Coordinator of the ward as soon as possible.
- If serious imminent threat to/ from the patient, a staff member or the public arises during an escort staff are to contact the SJA or Police for assistance by calling 000 on the work provided smart phone by:



- Opening the Emergency+ app
- Tapping on the relevant icon e.g. 000 Emergency which will connect you to emergency services
- When asked for the location read out:
 - street location where available or
 - the three words displayed at the bottom of the screen in the 'My location' section.
- help will be sent to the exact location.
- On return to the ward the escort will hand over relevant details (e.g. triggers, description of events) to the treating team and complete the following documentation:
 - document the incident details in the patient's medical record including mental state immediately prior to absconding.
 - update RAMP
 - complete Datix CIMS

- The treating team/ on call Duty Psychiatrist will be informed, and a plan of action documented in the patient's medical record.
- Inform the patient's PSP/ carer/ next of kin and document in the patient's medical record.

9. Alcohol and other drugs management

Patients on the Opioid Substitution Therapy (OST) Program as an inpatient will need to return to the hospital for their OST dose for the period of leave. Under no circumstances will patients be provided take-away doses.

- Patients on OST or who use alcohol and other drugs will be provided psychoeducation around tolerance, risk of overdose and harm reduction, interactions with psychiatric medications.
- Consider providing naloxone where appropriate. Refer to FSFHG Naloxone prefilled take-home syringes (Prenoxad) and nasal delivery device (Nyxoid) Guideline.
- Request patient's returning from leave to inform staff of any alcohol or drug use whilst on leave due to potential for intoxication on return to the ward.
- Patient to be informed of potential for urine drug screen or breathalyser to be completed if appropriate on return from leave and possible impact on future leave.
- Consult with AOD Consultant Liaison or Dual Diagnosis service for advice and safety planning.

10. Medication management

10.1. Fiona Stanley Hospital

- The treating team will review the medications of patients going on overnight, weekend or extended leave to determine if medications are required to be supplied for use during leave:
 - All leave scripts for dispensing in hospital must be clinically screened by the ward Pharmacist during business hours, prior to being sent to the Pharmacy Dispensary.
 - Wherever possible, leave prescriptions should be organised by the Treating Team by 16:00hrs the day prior to leave.
- In some cases, it may be appropriate for the patient/ family/ significant other to leave the unit with prescriptions to cover the leave period for dispensing at a community pharmacy. In these cases:

- consider patient's financial ability to pay for medication prior to this arrangement.
- vouchers can be given to FSH mental health inpatients after a joint decision by the treating consultant and pharmacist. Quantities are restricted to 10 vouchers per month across the three wards and supply to patients beyond this number must be approved by the Head of Service.
- The Ward Pharmacist will supply a medication list for the patient to refer to during their leave period.
- Leave medications will be kept in the allocated Patient's Own Medication shelves, or, in the case of S4R/ S8 medication, will be locked in the S4R/ S8 cupboards, until the patient is leaving the ward.
 - Where appropriate, return patient's own medications to the patient/ family/ significant other for use over leave.
 - Where patients have been dispensed single patient-use items as an inpatient, and can continue use of the items during leave (e.g. nicotine inhalers, salbutamol inhalers, oral contraceptive pill) these must be labelled with directions for use by the pharmacist, prior to being given to patient for use on leave)

10.2. Fremantle Hospital and FSH Cockburn Health

- The treating team will review the medications of patients going on overnight, weekend or extended leave to determine if medications are required to be supplied for use during leave:
 - All leave requests must be clinically screened by the ward Pharmacist during business hours, prior to being sent to the Pharmacy Dispensary.
 - The prescriber will generate a medication list with information on the required quantity using Notifications and Clinical Summary (NACS) or eMed (from Clinical Workbench) and will print their name, sign and date the list.
 - Wherever possible, leave prescriptions should be organised by the Treating Team by 16:00hrs the day prior to leave.
- If a dose administration aid is required for the patient's leave, the medication list needs to be generated ward pharmacist notified by 14:00hrs the day before the leave. The dose administration aid will only be ready at 16:30hrs the following day.
- Leave medications will be dispensed at the hospital pharmacy prior to the patient going on leave, during Pharmacy business hours.
- Where patients are going on leave outside of Pharmacy business hours, the prescriber may dispense the leave medications as per the FH Medication Dispensing After Hours by A Medical Prescriber: Mental health.

- Where patients have been dispensed single patient-used items as an inpatient, with labelled directions, the items will be provided to the patient from the ward.
- A sufficient amount of nicotine replacement therapy may be provided to the patient from the ward imprest as deemed appropriate by the treating Doctor, Nurse or Pharmacist.
- Leave medications will be kept in the allocated Patient's Own Medication shelves, or, in the case of S4R/ S8 medication, will be locked in the S4R/ S8 cupboards, until the patient is leaving the ward.

10.3. Leaving the ward

- Nursing staff are responsible for ensuring that the patient leaves with the appropriate leave medications.

11. Compliance/performance monitoring

- The Nurse Unit Manager of the relevant area will monitor evaluate compliance with this policy through routine clinical incident review processes.
- The Service Directors will ensure an annual review of all reported missing person/AWOL incidents is conducted in line with the WA Health Mental Health Consumers who are Missing or Absent Without Leave Policy and these reports will be monitored by the Service 5 Safety Quality Risk Committee.

12. Related policy documents

WA Health:

- Mental Health Consumers who are Missing or Absent Without Leave Policy
- Best Practice Guideline: Mental Health Consumers who are Missing or Absent Without Leave
- [Safety Planning for Mental Health Consumers Policy MP 0181/24](#)
- [Safety planning procedures for mental health consumers](#)
- [Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#)
- [Clinical Incident Management Policy](#) 2019 (MP 0122/19)
- [Medicines Handling Policy](#) (MP0139/20)

SMHS:

- [Rights of patients in mental health](#) (SMHS: 164)

- [Granting and management of leave for mental health inpatients](#) (SMHS: 11)
- [Missing or suspected missing mental health patients](#) (SMHS: 47)
- [Discharge against medical advice](#) (SMHS:4)

FSFHG:

- Advance Health Directives, Advance Care Plans and Enduring Power of Guardianship (FSFH-HW-POL-0031)
- Anticipating and reducing the risk of unpredictable behaviours (FSFH-MENH-POL-0027)
- Clinical Documentation (FSFH-HW-POL-0039)
- Clinical Handover (FSFH-HW-POL-0035)
- Medication Administration (FSFH-HW-POL-0022)
- Clinical Risk Assessment and Management (FSFH-HW-POL-0017)
- Mental Health Patient Escort (off campus) [FSFH-MENH-POL-0020]
- Medication Dispensing After Hours by A Medical Prescriber: Mental health (FSFH-MENT-POL-0010)
- Naloxone prefilled take-home syringes (Prenoxad) and nasal delivery device (Nyxoid) (FSFH-HW-GUI-0029)
- Patient initiated early discharge (FSFH-HW-GUI-0039)

13. Related standards

- NSQHS standards:
 - Medication Safety
 - Comprehensive Care
 - Communicating for Safety
- Chief Psychiatrists Standards for Clinical Care:
 - Assessment
 - Care Planning
 - Risk assessment and management
 - [Chief Psychiatrist's Guidelines for the sexual safety of consumers of Mental Health Services in WA](#)

14. Related legislation

[The Mental Health Act 2014](#)

15. Authorisation

EXECUTIVE SPONSOR: Service Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	05/2016	CNS, CI	FH MHS Clinical Governance Committee	FH Policy Committee	05/2019
2	06/2018	Nurse Director, Service 5; CNS, Clinical Improvement	MHS Safety Quality Risk Committee	FSFHG Policy Committee	06/2021
3	12/2020	Nurse Director, Service 5; CNS, Clinical Improvement	MHS Safety Quality Risk Committee	FSFHG Policy Committee	12/2022
4	12/2022	CNS, Clinical Improvement	DTC,	Service 5 Safety Quality and Risk Committee	01/2025
4.1	08/2023	CNS Clinical Improvement	Minor update, no change to review date	Service 5 Safety Quality and Risk Committee	01/2025
5	07/2024	CNS Clinical Improvement		Service 5 Safety Quality and Risk Committee	07/2027